Case study: Lauren

Condition: **Distal radius fracture – left fracture, closed reduction and cast**

37-year-old woman · Outpatient fracture clinic, metropolitan public hospital

Lauren Hughes is a 37-year-old right-handed woman with a **left distal radius fracture** who attends the outpatient fracture clinic at a metropolitan public hospital. Lauren has worked as a hairdresser at a local salon for twelve years, usually holding the comb and sectioning hair with her left hand while cutting with her right. She lives with her partner, Craig, and their two primary-school-aged children. One week ago, she presented to the emergency department, where her fracture was treated with closed reduction and a below-elbow cast. She missed her first fracture clinic appointment four days ago, and no reason is recorded.

Lauren's X-ray this morning shows that the fracture position has been maintained, and the orthopaedic registrar has documented "cast satisfactory, continue". Her fingers are pink, warm and move well, and she reports only mild aching. You have been asked to review her cast comfort, give advice on managing everyday tasks and discuss when she might return to work.

Craig accompanies Lauren into the room. He is friendly and helpful, carries her bag and jokes that she is "the clumsiest person I know". When you explain that part of the appointment is usually done one-on-one, he smiles and says, "I'll stay. She forgets everything the doctors tell her." When you ask Lauren directly how the fall happened, Craig answers before she can, explaining that she "missed the back steps carrying the washing". The ED triage note records that Lauren said she "tripped on the stairs inside".

Lauren is quiet and mostly looks at the floor. She asks whether she will be able to style hair with the cast on and whether a cast cover would let her wash clients' hair. She mentions that the edge of the cast rubs near her thumb. When you ask whether she can manage showering and dressing at home, she says, "Craig helps," and looks back down. Her phone buzzes several times during the appointment, and each time she checks it immediately. When you ask whether time off work is causing any financial worries, she glances at Craig and says, "Money's Craig's department."

The room is warm, and Lauren takes off her cardigan while you check her cast. You notice a row of four oval, fingertip-sized bruises on the inside of her right upper arm, some purple and others yellow-green. She quickly pulls the cardigan back on and says, "I bruise like a peach." When you gently ask whether her arm is sore, she shakes her head and does not answer. Craig says, "She's always walking into things."

There is no documentation of whether Lauren was asked any routine safety screening questions in ED, whether these bruises were noted at that time or whether Lauren has had previous injury presentations. You do not know why she missed her first appointment, how the children usually get to and from school or who else Lauren has for support. The only contact number recorded on her file is Craig's mobile.

Craig looks at his watch and stands. "We'll have to wrap this up. School pick-up's in fifteen minutes and it's right across town. Can we just grab one of those cast covers on the way out? We'll book the next appointment ourselves once things settle down." He holds Lauren's bag out to her, and she begins to stand.

You are the occupational therapy student reviewing Lauren in the fracture clinic and your clinical supervisor is not currently in the room.

Present Lauren's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

Unofficial practice material. All cases are fictional and written for ISBAR communication practice; they are not clinical guidance. Not produced by or affiliated with any university.